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/ **Practice Exam - 1**

Correct Answer

Next Question

Practice Exam - 1

Question 52 of 117



An 18-year-old woman is evaluated 2 weeks after an uncomplicated vaginal delivery for progressive dyspnea, orthopnea, and lower-extremity edema. She had no chest pain. She had no prior medical problems and an unremarkable pregnancy. She is not breastfeeding. Family history is notable for her mother having developed heart failure in her early 40s and was known to have a left ventricular ejection fraction of 25% before initiation of guideline-directed medical therapy. She has 3 sisters and one brother who do not have any known cardiac conditions. The patient has no history of alcohol or illicit drug use.

Physical examination reveals BP 115/80 mm Hg, HR 94, JVP 12 cm, an S3 gallop, and mild peripheral edema. Laboratory studies show normal renal function and no evidence of infection or thyroid disease. ECG revealed normal sinus rhythm with a QRS 150 msec and LBBB. Transthoracic echocardiography shows a globally dilated left ventricle with a left ventricular ejection fraction of 30%.

In addition to initiating quadruple guideline-directed medical therapy, which of the following would you recommend?

A. Bromocriptine



B. Ivabradine



C. Refer to a genetic counselor for cardiomyopathy genetic testing

D. Evaluate for myocarditis with endomyocardial biopsy

E. Refer to EP for CRT-D

Commentary

References

Correct Answer: C. Refer to a genetic counselor for cardiomyopathy genetic testing

Teaching Point

AHFTC cardiologist should have the skill to oversee genetic testing of patients with cardiomyopathy, including selection of patients and collaboration with genetic counselors.

Explanation

The patient has peripartum cardiomyopathy and a clear family history of HFrEF (mother). A seminal study in the NEJM demonstrated that patients with peripartum cardiomyopathy can carry a genetic variant associated with cardiomyopathy, with titin being the most common. Referral to genetic counselor will facilitate patient education, informed consent, and ultimately cascade screening should a variant be identified. The patient has many relatives who potentially could benefit from cascade testing including her child and 4 siblings, and any of their children.

Incorrect Answers

1. Bromocriptine: Bromocriptine has shown benefit in selected patients with severe PPCM but is not currently routine standard therapy in U.S. practice. Referral to be enrolled in the REBIRTH trial (NCT05180773) would have been a valid response, if offered.
2. Ivabradine should not be considered until the heart rate is known after maximally tolerated beta-blockers.
3. Myocarditis/endomyocardial biopsy – the clinical scenario is more consistent with peripartum cardiomyopathy than myocarditis. If there was clinical suspicion for myocarditis, the next step in evaluation would be a cardiac MRI rather than endomyocardial biopsy, as myocarditis is often initially evaluated noninvasively with cardiac MRI and the patient does not have a clinical course of hemodynamic/electrical instability warranting endomyocardial biopsy at this point,
4. Refer to EP for CRT-D: CRT-D is only indicated if the patient continues to have reduced LVEF and the LBBB after a trial of GDMT.

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